

جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : صبحيه محمد السيد عبد الحميد

المحافظة : الفيوم الحي / المدينة : يوسف الصديق العنوان : كحل

رقم البطاقة : 28309222301005 رقم التليفون : 01146970466 تاريخ تسجيل الإستمارة : 05-10-2026

العمر(تاريخ الميلاد) : 22-09-1983 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري على العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة:

الوظيفة:

الوظيفة:

التوقع:

التوقع:

التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: صبيحه محمد السيد عبد الحميد National ID: 28309222301005	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 صبحية
 محمد السيد عبد الحميد
 كحك بحري
 مركز يوسف الصديق - الفيوم
 ٢٠٢٣/٠٩/٢٢ ٢٨٣.٠٩٢٢٢٣.١٠.٠٥
 JU8492765

٢٠٢٣/٠٩/٢٢ ٢٨٣.٠٩٢٢٢٣.١٠.٠٥
 ربة منزل
 انثى
 مقيمة
 خليل عبد العاطي علي عبد الكريم
 البطاقة سارية حتى ٢٠٣٠/٠٩/١٧
 ٦

Patient Name: Sobheya Mohamed Al Sayed Abd Al Hamid	Date : 25/09/2024
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 3226/2024	Pat. ID: 79464

**POST CONTRAST MULTI-SLICE CT SCAN OF THE
CHEST, ABDOMEN, AND PELVIS: -**

Findings:

I Chest:

- Left breast LOQ irregular lesion measuring 5.7 X 3 cm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size showing bilobar hypoe-enhancing focal lesions, the largest seen at segment IVb measures about 2x2 cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Name: <u>صبيحة محمد السيد عبد الحميد</u>		
MR. : <u>4509</u>		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient has metastatic breast cancer (MBC) with liver involvement, estrogen receptor (ER) 8/8 positive, progesterone receptor (PR) 8/8 positive, and HER2 2+ not amplified. The current treatment regimen includes an aromatase inhibitor (AI) combined with Zoladex, Femara, calcium supplementation, and one alpha-Zoladex 10.8 mg subcutaneously for continuation therapy. The plan is to continue this regimen as outlined.		
تحريراً في ,		
التاريخ :	الوقت :	18-09-2026
الطبيب المعالج :	ايه مدحت بدوي محمد	
يعتمد ,		



Patient Name	: السيد / صبرحة محمد السيد عبد الحميد	Request No.	: 110111611
Patient No.	: 28309222301005	Request Date	: 25/08/2024
Path No.	: 6296/2024	Verification Date	: 03/09/2024
Sex	: Female	Financial Category	: مبادرة
Age	:	Department	: مبادرة أورام ندي - سوهاج
Referring Doc	: Maro		

Pathology Report

CLINICAL DIAGNOSIS: LI breast LOQ mass.

NATURE OF SPECIMEN: TCNB.

GROSS EXAMINATION : The specimen consists of 4 tissue cores (1.7cm long), grayish in color, totally submitted in one cassette.

MICROSCOPIC EXAMINATION: Sections revealed infiltration by atypical epithelial cells arranged in cords, nests and sheets, the cells show pleomorphism, hyperchromatic nuclei and rare mitosis, the stroma is desmoplastic.

DIAGNOSIS:

Labeled as LI breast LOQ mass, TCNB:

- Features of invasive duct carcinoma, NST, GII.
- Ductal carcinoma in-situ component is of intermediate grade & cribriform pattern.
- Foci of Lymphovascular Invasion were detected.
- Tumor Infiltrating Lymphocytes is of moderate intensity.

Signature

Pathologist: Dr. Ghada Mostafa

Dr. Cherry Gamal

N.B : Specimens received at the department of pathology are examined and sampled for diagnostic purposes. The specimens are normally held for four weeks and then disposed. A request for return of the specimens must be made at the time of surgery or within four



Patient Name : السيد / صبيحة محمد السيد عبد الحميد
Patient No. : 28309222301005
Path No. : 6296/2024
Sex : Female
Age :
Referring Doc : Maro

Request No. : 110111611
Request Date : 25/08/2024
Verification Date : 03/09/2024
Financial Category : مبادرة
Department : مبادرة أورام ندى - سوهاج

Pathology Report

CLINICAL DIAGNOSIS: Lt breast LOQ mass.

NATURE OF SPECIMEN: TCNB.

GROSS EXAMINATION : The specimen consists of 4 tissue cores (1.7cm long), grayish in color, totally submitted in one cassette.

MICROSCOPIC EXAMINATION: Sections revealed infiltration by atypical epithelial cells arranged in cords, nests and sheets, the cells show pleomorphism, hyperchromatic nuclei and rare mitosis, the stroma is desmoplastic.

DIAGNOSIS:

Labeled as Lt breast LOQ mass, TCNB:

- Features of Invasive duct carcinoma, NST, GII.
- Ductal carcinoma in-situ component is of intermediate grade & cribriform pattern.
- Foci of Lymphovascular invasion were detected.
- Tumor infiltrating Lymphocytes is of moderate intensity.

Signature

Pathologist: Dr. Ghada Mostafa
Dr. Cherry Gamal

N.B : Specimens received at the department of pathology are examined and sampled for diagnostic purposes. The specimens are normally held for four weeks and then disposed. A request for return of the specimens must be made at the time of surgery or within four

Patient Name: Sobheya Mohamed Al Sayed Abd Al Hamid	Date : 25/09/2024
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 3226/2024	Pat. ID: 79464

**POST CONTRAST MULTI-SLICE CT SCAN OF THE
CHEST, ABDOMEN, AND PELVIS: -**
Findings:

I. Chest:

- Left breast LOQ Irregular lesion measuring 5.7 X 3 cm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II. Abdomen & pelvis:

- The liver is of average size showing bilobar hypoe-enhancing focal lesions, the largest seen at segment IVb measures about 2x2 cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
في المراكز الطبية المتخصصة
مستشفى أروام دار السلام
إدارة الأشعة

Patient Name: Sobhaya Mohamed Al Sayed Abd Al Hamid	Date : 25/09/2024
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 3226/2024	Pat. ID: 79464

- The scanned skeleton shows:
 - L4 bilateral pars break.
 - No worrisome lytic or sclerotic lesions.

Opinion:

- Multiple hepatic focal lesions likely of metastatic nature.
- No evidence of pulmonary metastatic deposits.

Dr. Shaimaa Megahed, MSc

Dr. Mohamed Mohsen, M.D.

Patient Name: Sobheya Mohamed Al Sayed Abd Al Hamid	Date : 20/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 42 Year(s)
Medical ID:	Pat. ID: 79464

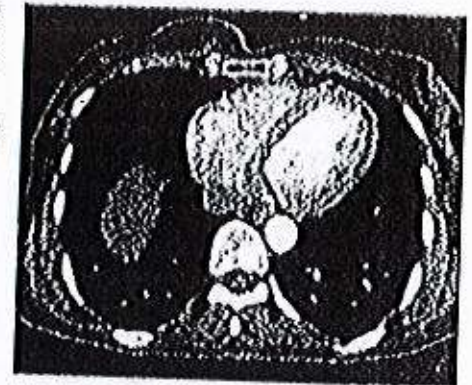
POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- o Known case of left breast cancer, compared to the previous study report dated 6-2-2025, the current study revealed:

Findings:

- Regressive course of the Left breast LOQ irregular lesion measuring 3x1 cm (compared to 4.3 x 1.5 cm).???
- No sizable pulmonary nodules, masses or areas of consolidation.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.
- Normal CT appearance of the mediastinal vessels.



OPINION:

- Stationary course of the disease process.

Marina Essam, MSc
Dr.Noura Nabil,MD

MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: -

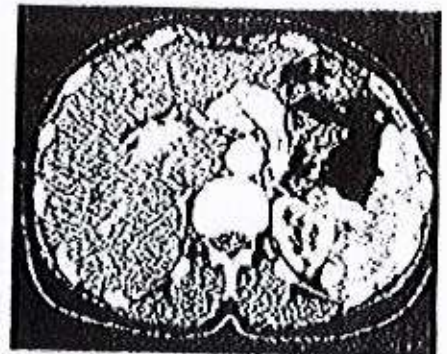
Patient Name: Sobheya Mohamed Al Sayed Abd Al Hamid	Date : 20/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 42 Year(s)
Medical ID:	Pat. ID: 79464

Clinical History:

- Known case of left breast cancer, compared to the previous study report dated 6-2-2025, the current study revealed:

Findings:

- The liver is of average size still showing bilobar hypodense ill-defined multiple bilobar focal lesions, the largest seen at segment IVb measures about 12x17 mm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size (12.5 cm) and parenchymal texture of the spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.
- IUD is noted.
- Still noted L4 bilateral pars break.



Opinion:

- Stationary course of the disease process.

Marina Essam, MSc
Dr. Noura Nabil, MD

Signature Noura Nabil
09/08/2025 6:48 PM



Dr. M. J. Jayasinghe
Medical Officer
Pathology

Request No. : 110111011
Request Date : 25/02/2024
Verification Date : 03/03/2024
Financial Category : 1
Department : Pathology

Request No. : 110111011
Request Date : 25/02/2024
Verification Date : 03/03/2024
Financial Category : 1
Department : Pathology

Pathology Report

1. CLINICAL HISTORY: Left breast lump mass.

2. SPECIMEN: TCMs.

3. EXAMINATION: The specimen consists of 4 tissue cores (1.7cm long), grossly examined and submitted in one cassette.

4. MICROSCOPIC EXAMINATION: Sections revealed infiltration by atypical epithelial cells, forming cords, nests and sheets, the cells show pleomorphism, hyperchromatic nuclei and mitoses, the stroma is desmoplastic.

- 5. FINDINGS: Left breast lump mass, TCMs.
- 6. Diagnosis: Invasive duct carcinoma, NST, CR.
- 7. The carcinoma in-situ component is of intermediate grade & cribriform pattern.
- 8. Lymphovascular invasion was detected.
- 9. Infiltrating lymphocytes is of moderate intensity.

10. Ref: Dr. Ananda Mestana
11. Ref: Dr. Ananda Mestana

12. Specimens received at the department of pathology are examined and reported on the same day. The specimens are normally held for four weeks and then disposed of. A request for return of the specimens must be made at the time of surgery or within four weeks.



Official stamp
 Date: 10/09/2017
 Signature: [Signature]

Request No. 110141611
 Request Date 25/08/2017
 Verification Date 10/09/2017
 Financial Category 1
 Department Pathology

Request No. 110141611
 Request Date 25/08/2017
 Verification Date 10/09/2017
 Financial Category 1
 Department Pathology

Pathology Report

1. CLINICAL HISTORY: Left breast LOQ mass.

2. PREVIOUS SPECIMEN: TGNB.

3. GROSS EXAMINATION: The specimen consists of 4 tissue cores (1.7cm long), grayish in color, submitted in one cassette.

4. MICROSCOPIC EXAMINATION: Sections revealed infiltration by atypical epithelial cells, forming cords, nests and sheets, the cells show pleomorphism, hyperchromatic nuclei, mitotic figures, and the stroma is desmoplastic.

- 5. Final diagnosis: Left breast LOQ mass, TGNB.
- 6. Invasive duct carcinoma, NST, GII.
- 7. In situ carcinoma component is of intermediate grade & cribriform pattern.
- 8. Lymphovascular invasion was detected.
- 9. Inflammatory Lymphocytes is of moderate intensity.

10. Pathologist: Ghada Mostafa
 Pathology General

Specimens received at the department of pathology are examined and sampled for diagnostic purposes. The specimens are normally held for four weeks and then disposed. A written report of the specimens must be made at the time of surgery or within four weeks.